

What the payer does to the schedule

The same operational event carries a different consequence in every payer class

- Intake

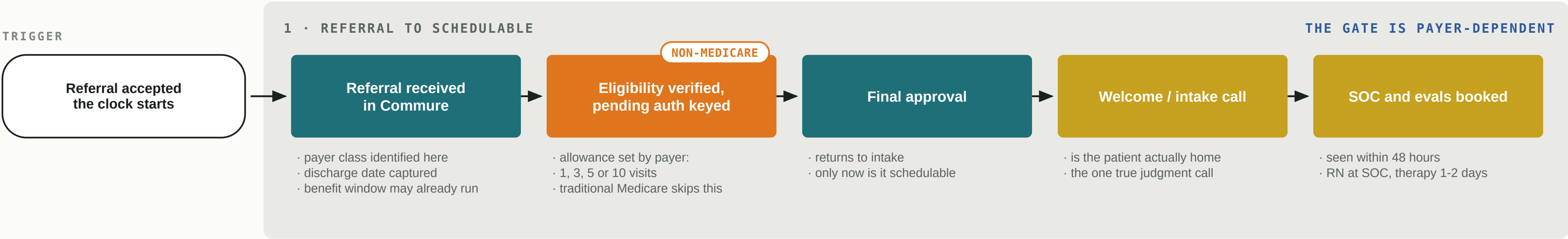
Insurance & Auth

PCC / Scheduler

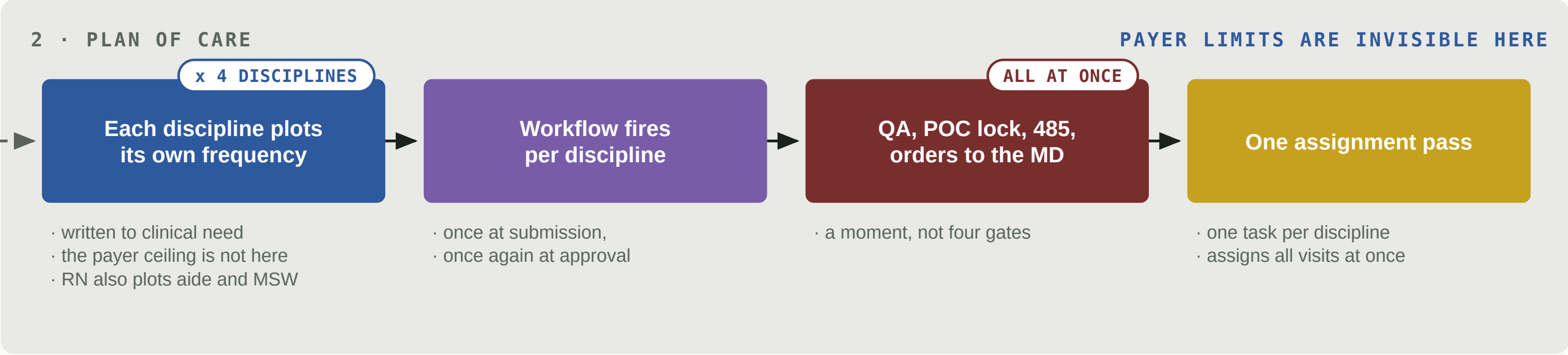
Clinician
- DCS

HCHB

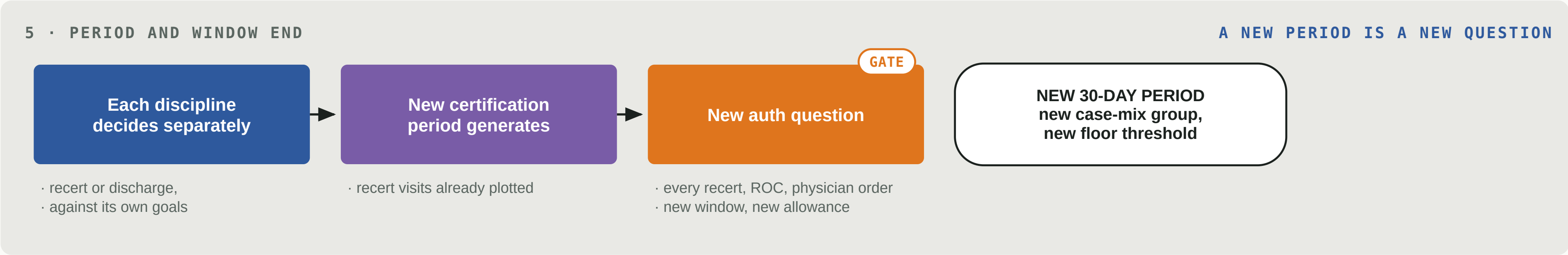
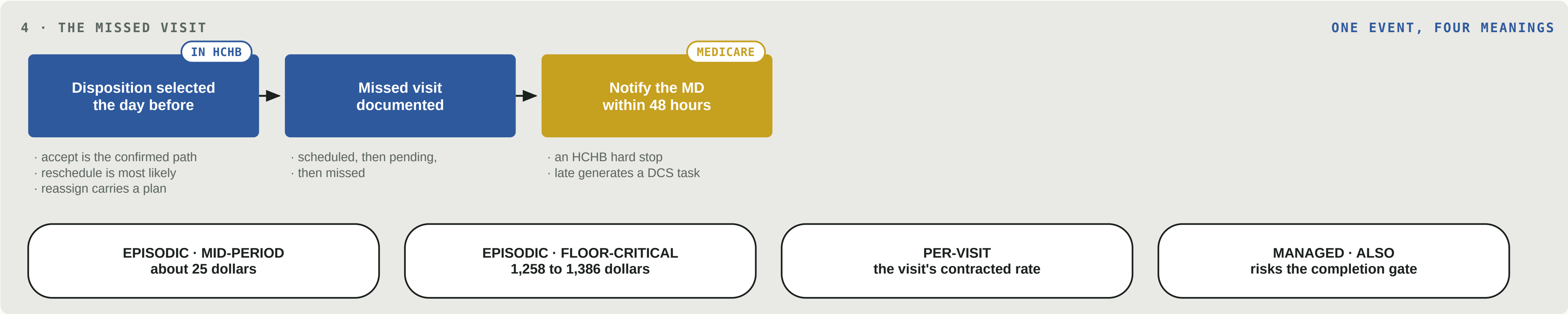
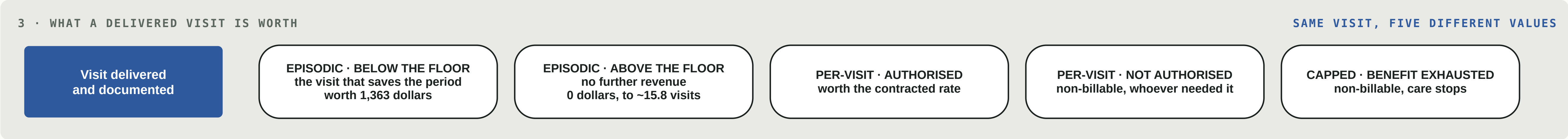
TRIGGER



Payer rule already exists
in a coordination note,
written days earlier



White outline =
a payment state,
not an actor



THE FOUR CEILINGS ON ONE EPISODE – INDEPENDENT TESTS, NOT ONE RULE

PERMISSION

- what the payer allows
- unauthorised is non-billable
- backdating window 0-5 days

FLOOR

- LUPA, 2 to 5 visits by group
- below it the period reprices
- a cliff, not a slope

CEILING

- visits above the floor
- earn nothing to ~15.8 visits
- no one sees this today

CAP

- the annual benefit limit
- commercial and Medicaid
- the employer may set it

WHERE IT BREAKS

- The payer rule is written days before the plan of care
- and never surfaces at it
- Pending-auth visits sit on no calendar, count toward nothing
- Period state is invisible, so every miss is priced the same
- About 50 pending-auth notices a day train bulk-clearing
- A rebooked visit spends two slots to deliver one